

Nova Surgical Institute

6640 VAN NUYS BLVD, #101,
LOS ANGELES CA, 91405
(818) 405-0064

PROCEDURE NOTE

Date: 12/06/2024

RE: JUAN LUIS JUAREZ CEJA

DOB: 07/25/1978

Preoperative diagnosis:

1. Bilateral L4-L5, L5-S1 lumbar disc displacement.
2. Lumbago.

Postoperative diagnosis:

1. Bilateral L4-L5, L5-S1 lumbar disc displacement.
2. Lumbago.

Procedure Performed:

1. Bilateral L4-L5, L5-S1 lumbar transforaminal epidural injection under fluoroscopy.
2. Fluoroscopy supervision of the lumbar spine.
3. AP and Lateral views of the lumbar spine.

Surgeon:

David Gostine

Informed Consent:

The procedure was described to the patient and the possible benefits, side effects and complications were discussed in detail. The patient was also offered alternative forms of treatment. The patient after accepting the possible complications signed an informed consent form.

Anesthesia:

Anesthesia services were provided in accordance with the patient's request.

Position:

The patient was positioned in the prone position on the fluoroscopy table. The lower back area was prepped draped in a sterile manner.

Procedure of Lumbar Transforaminal Epidural Injection:

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The fluoroscopy unit was turned to a left oblique fashion to identify the superior and dorsal aspect of the left lumbar L4-L5 neural foramina. A 27-gauge 3 1/2 inch spinal needle was inserted into the superior and dorsal aspect of the left L4-L5 neural foramina. A 1/2 cc. of Omnipaque 240 dye was injected to outline the epidural space. 2cc of mixture of 0.5% Lidocaine mixed with Dexamethasone was injected. The needle was withdrawn.

The same procedure was repeated at the L5-S1 level(s) on the left side.

The procedure was then repeated at the L4-L5, L5-S1 level(s) on the right side.

A total of 8mg of Dexamethasone was injected.

The patient did not complain of any discomfort or radicular pain during the needle placement or at the time of injection of the dye or anesthetic. The needle was withdrawn.

The patient tolerated the procedure well without any complications and was taken to the recovery room in a stable condition for a 30 minute observation. The vital signs remained stable; there were no sensory or motor changes in the lower extremities.

Plan:

Follow-up in two weeks for an evaluation of the procedure.

Sincerely,

Electronically Approved by:



David Gostine

DOD: 12/06/2024

DOT: 12/06/2024